

Postpartum Recovery and Feeding Guide

Client Education Handout | Jake Thornton, CNM | AMWA-Affiliated Midwifery Practice

This handout provides general education about the postpartum period. It is not a substitute for individual clinical guidance. Every birth and recovery is different. If you have questions or concerns at any point in your postpartum period, contact the practice.

The First Two Weeks

The first two weeks after birth are a time for rest, close observation of your physical recovery, and establishing feeding. Your body has done extraordinary work. Protect your recovery by keeping your activity proportional to your bleeding - if bleeding increases when you do more, slow down.

Physical recovery: Expect vaginal bleeding (lochia) for 4-6 weeks. In the first week, lochia is red and may contain small clots no larger than a quarter. It gradually transitions to pink or brown, then to a yellow-white discharge. Heavy gushing or soaking more than one pad per hour in the first two weeks is not normal - contact the practice.

Perineum care (vaginal birth): Apply an ice pack wrapped in a cloth for 10-20 minutes every 2-3 hours for the first 48 hours to reduce swelling. Use a peri bottle with warm water to cleanse after voiding. Sitz baths (plain warm water, 15-20 minutes) ease discomfort and support healing from day two onward. Witch hazel pads can reduce swelling and discomfort.

Cesarean incision care: Keep the incision dry for the first 48 hours. After that, gently clean with mild soap and water and pat dry. Avoid submerging the incision in water until fully healed (typically 4-6 weeks). Watch for redness, warmth, separation, or discharge from the incision - these require evaluation.

Nutrition and hydration: Eat regularly and prioritize iron-rich foods (red meat, leafy greens, lentils, fortified cereals) to support recovery from blood loss. If breastfeeding, aim for at least 64 oz of water daily and increase caloric intake by approximately 300-500 calories above your pre-pregnancy baseline.

Weeks Three Through Six

By week three, most clients notice a significant improvement in energy and a reduction in discomfort. This is the most common time to overdo activity - feeling better does not mean fully healed.

Activity: Short walks are encouraged from week one. Avoid lifting anything heavier than your baby until week six. Do not return to exercise, running, or core-intensive activity until cleared at your six-week visit. Pelvic floor physical therapy is recommended for all postpartum clients, regardless of delivery type.

Mood and mental health: Baby blues - tearfulness, mood swings, and emotional sensitivity - are common in the first two weeks as hormones shift dramatically. Baby blues typically resolve by day 14. If low mood, excessive anxiety, difficulty bonding, or thoughts of self-harm persist beyond two weeks, or emerge at any point in the first year, contact the practice. Postpartum mood disorders are common, treatable, and not a reflection of your parenting.

Contraception: Ovulation can return as early as three to four weeks postpartum, even before your period returns and even if you are breastfeeding. If you do not wish to conceive, discuss contraception at the two-week visit. Lactational amenorrhea method (LAM) requires exclusive breastfeeding, no supplemental feeds, and the absence of menstruation - and is only reliable for the first six months.

Feeding Guide

Breastfeeding

Breast milk supply is established in the first 10-14 days. Frequent, effective removal of milk - through nursing, pumping, or both - is the signal that drives supply. Feed on your baby's cues: early hunger signs include rooting, hand-to-mouth movement, and increased alertness. Crying is a late hunger cue.

- Aim for 8-12 feeds per 24 hours in the first two weeks
- Ensure a deep, comfortable latch - the baby's mouth should cover the areola, not just the nipple
- Expect some initial nipple tenderness; severe pain, cracking, or blistering warrants a lactation evaluation
- Track feeds and output in the first week: 6-8 wet diapers and 3-4 yellow stools per day by day five indicates adequate intake

Combination Feeding

Combination feeding - using both breast milk and formula - is a valid and common choice. If introducing a bottle, consider waiting until breastfeeding is well established (typically 3-4 weeks) to minimize nipple preference concerns, unless supplementation is medically indicated earlier. Use paced bottle feeding: hold the bottle horizontal, allow the baby to actively suck rather than letting milk flow freely, and pause the feed periodically.

Warning Signs - When to Contact the Practice

Call Immediately (or call 911)

- Heavy vaginal bleeding: soaking more than one pad per hour for two or more consecutive hours
- Fever of 100.4°F (38°C) or higher
- Severe headache, vision changes (spots, blurring), or upper right abdominal pain
- Chest pain or difficulty breathing
- Thoughts of harming yourself or your baby
- Signs of blood clot: calf pain, redness, swelling (one leg more than the other)

Call Within 24 Hours

- Worsening perineal or incision pain after day five
 - Signs of mastitis: localized breast redness, hardness, warmth, with fever
 - Persistent baby blues beyond two weeks
 - Difficulty with breastfeeding that is not improving with positioning adjustments
 - Newborn: fewer than 6 wet diapers by day five, jaundice deepening, difficulty waking to feed
-

Your Six-Week Appointment

The six-week postpartum visit covers physical recovery assessment, contraception counseling, pelvic floor screening, mood and wellbeing evaluation, and feeding review. Bring your questions - this visit is yours. Consider writing them down in advance. Topics to consider: return to exercise, resuming sexual activity, contraception, mood, infant feeding, and any concerns that developed during the recovery period.

Community Resources - Atlanta Area

- Atlanta Maternal Wellness Alliance (AMWA): peer support groups and community workshops - amwa-atlanta.org
- Georgia WIC Program: lactation consultants and nutrition support - available at all Atlanta-area WIC offices
- ICAN of Atlanta: cesarean education and support group
- Postpartum Support International warmline: 1-800-944-4773